Ysbyty Gwynedd
Anaesthetics Department
Adran Anaestheteg

CRITICAL CARE

YG CG 1009 MAY 2021.

Guidance for the management of patients with suspected or confirmed *Clostridium difficile* infection

REVIEW BY (MAY 2024)

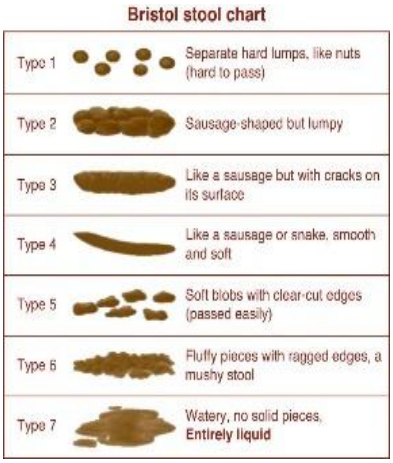
Author(s): Eirlys Walker
Reviewer: Guidelines Review group

Key Points

- Patients who are suspected of having *Clostridium Difficile* infection should be isolated immediately.
- Patients are considered infected when the bacterium produces toxins
- An integrated care pathway (ICP) should be commenced on all patients who test positive for C.Diff toxin
- For further guidance visit: IPC 1 - CDI protocol v7 FINAL Aug' 2019.pdf (wales.nhs.uk)

INTRODUCTION

Clostridium difficile (C.difficile, CDiff) is an anaerobic, spore forming, gram-positive bacterium that is found in the large intestine. When the balance of normal gut flora is disturbed, for example by frequent use of antibiotics, in particular cephalosporins and quinolones, it has the potential to cause illness. Toxins produced by C. difficile damage the large bowel causing watery, explosive, foul smelling diarrhoea. A percentage of healthy adults are colonised with the bacterium, where they carry it in their guts but it rarely causes any problems. However, colonised individuals have the potential to become toxin positive and develop symptoms of infection. Symptoms can range from a few bouts of diarrhoea in mild cases to more severe and potentially life-threatening complications such as pseudomembranous colitis, toxic megacolon and even intestinal perforation. Patients who develop unexplained diarrhoea (Bristol Stool Type 5-7) should raise suspicion for C.Diff and more than 2 bout of diarrhoea in a 24 hours period should trigger isolation and testing.



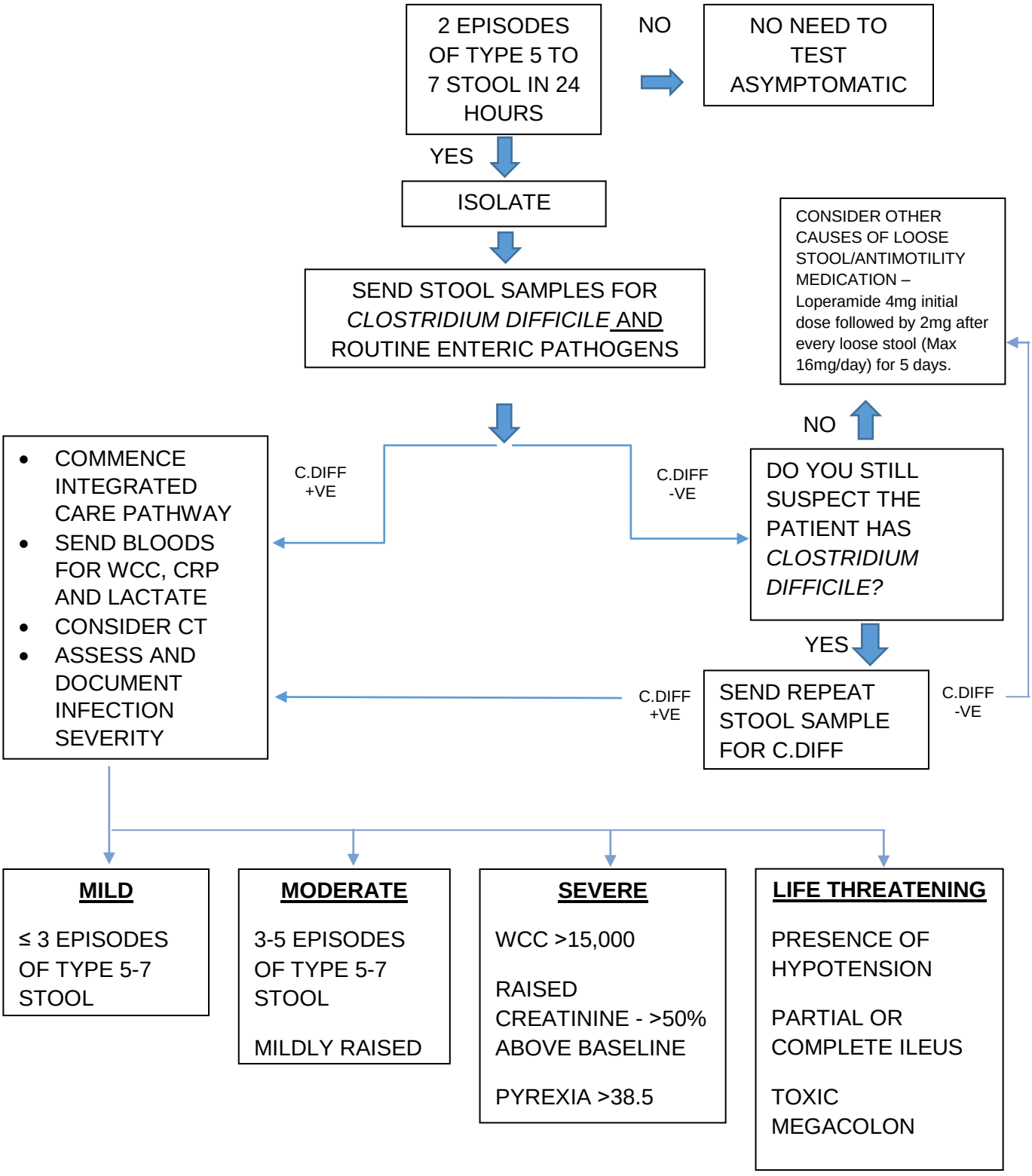
TESTING

Testing for C.Diff is in 2 parts:

PCR detection: This indicates the presence of a clostridium that is capable of producing the C.Diff toxin. PCR may be detected in the absence of disease i.e. carrier state. (Reported as GDH +ve)

Toxin detection: This indicates the production of toxins by actively dividing bacteria and is diagnostic for *Clostridium Difficile Disease* (Reported as Toxin +ve)

Carrier patients will be GDH +ve, Toxin –ve, where patients with CDD will be GDH +ve, Toxin +ve.



REVIEW MEDICATION

- CONSULT MICROGUIDE TO START APPROPRIATE TREATMENT (Oral VANCOMYCIN 125mg 6hrly for 10-14 days in non-severe and 14 days in severe)
- STOP ALL OTHER ANTIBIOTICS UNLESS BEING USED TO TREAT SEPSIS ELSEWHERE
- REVIEW OF PPI AND IF POSSIBLE USE RANITIDINE IF NEED FOR ACID REGULATION
- DON'T USE ANTIMOTILITY MEDICATION

DAILY REVIEW OF PATIENTS TO INCLUDE DOCUMENTING DISEASE SEVERITY, MONITORING FOR FEVER, NEWS SCORE, HEART RATE, CRP, PRESENCE OF ABDOMINAL PAIN, FLUID AND ELECTROLYTES AND NUTRITIONAL STATUS.